# Prodigy Anesthesia Quick Review

## Insulinoma

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| What is an insulinoma? | An insulinoma is a neoplasm of the beta cells of the pancreas.<br><br>*Longnecker DE, Newman MF, Brown DL, Zapol WM. Anesthesiology. 2nd ed. New York: McGraw-Hill; 2012: 1044.* |
| What are the principal manifestations of an insulinoma? | An insulinoma causes hypersecretion of insulin which results in severe hypoglycemia and its sequelae including depression, seizures, and coma.<br><br>*Nagelhout JJ, Plaus KL. Nurse Anesthesia. 5th ed. St. Louis, MO: Elsevier Saunders Company; 2014: 786.* |
| What is the primary treatment for an insulinoma? | Treatment for an insulinoma is surgical excision of the metastatic tumor. In advanced cases, the entire distal pancreas may have to be excised. This is referred to as a Child procedure.<br><br>*Nagelhout JJ, Plaus KL. Nurse Anesthesia. 5th ed. St. Louis, MO: Elsevier Saunders Company; 2014: 786.* |
| What medical treatment may be used as a temporizing measure prior to surgical excision of an insulinoma? | Patients are often managed preoperatively with diazoxide which inhibits insulin release from beta cells.<br><br>*Hines RL, Marschall KE. Anesthesia & Co-Existing Diseases. 6th ed. Philadelphia, PA: Elsevier-Saunders; 2012: 385.* |
| What is the primary anesthetic goal for the management of a patient undergoing surgical excision of an insulinoma? | The primary goal is maintaining a normal blood glucose level. Surgical manipulation of the tumor may result in profound hypoglycemia. Hyperglycemia may result rapidly after excision of the tumor. It is accepted that glucose-containing fluids should be administered intravenously during the surgery as many of the symptoms of hypoglycemia (diaphoresis, tachycardia, and hypertension) may not be noticeable during anesthesia. Blood glucose levels should be checked every 15 minutes during the procedure.<br><br>*Hines RL, Marschall KE. Anesthesia & Co-Existing Diseases. 6th ed. Philadelphia, PA: Elsevier-Saunders; 2012: 385.* |

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